

Key Indicators:

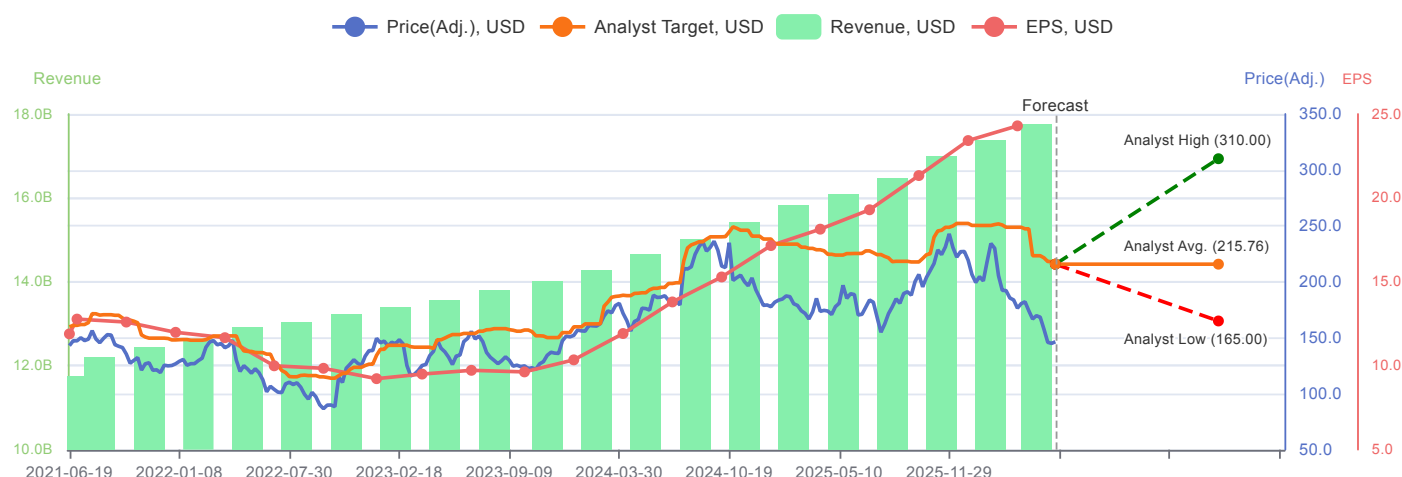
🕒 Date: Jun 20, 2026

Stock Price

\$141.2

52-Week Range	\$140.1 - \$245.5	EPS Revisions (90d)	↑ 3 ↓ 6	Revenue	\$17.8B
Market Cap	\$8.5B	PEG Ratio	0.16	Revenue Forecast	\$18.6B
P/E Ratio	5.62	FCF Yield	10.7	1-Year Change	-17.4%
P/E (Fwd.)	6.08	EV / EBITDA	5.03	Div Yield	0.57%
EPS Actual	21.7	Book / Share	122.2	Div. Growth Streak	-
EPS Estimate	23.2	Beta (5Y)	1.08	Next Earnings	2026-07-27

5-Year Chart



Executive Summary

Warren AI

Universal Health Services, Inc. (NYSE: UHS) is a prominent U.S. healthcare facilities operator managing both acute care hospitals and behavioral health facilities, serving patients across the United States, United Kingdom, and Puerto Rico with a network of over 400 facilities and trailing twelve-month revenue of approximately **\$17.8 billion**.

Q1 2026 results beat expectations, with EPS of \$5.62 (vs. \$5.44 forecast) and revenue of \$4.49 billion (vs. \$4.39 billion forecast), representing **9.6% revenue growth and 16.1% EPS growth year-over-year**. Operating cash flow reached \$402 million, up 11.7% from Q1 2025. Acute care same-facility revenue rose 8.2% and behavioral health same-facility revenue increased 7.3%, though volumes were impacted by approximately 200 bps in acute care and 40–50 bps in behavioral health due to weather and weaker flu activity.

A key strategic development is the **pending acquisition of Talkspace**, a virtual behavioral health platform with 6,000 licensed professionals, funded in part by a \$900 million credit facility expansion. Management expects the deal to be *accretive within 12 months* and to reach a single-digit EBITDA multiple by year three. UHS is also deploying AI across eight revenue cycle use cases and expanding clinical AI initiatives for 2026.

On the bearish side, Raymond James downgraded UHS to Market Perform, citing concerns about the attainability of Q2–Q4 EBITDA guidance. The stock has declined approximately 19% over the past 12 months and 29% year-to-date, trading near its 52-week low. Core EBITDA growth, excluding Medicaid supplemental payments and seasonal headwinds, was in the low single digits in Q1. The company also faces a projected **\$75 million pre-tax HIX headwind** in 2026, looming Medicaid supplemental payment reductions of \$360–\$400 million annually from 2028–2032, and ongoing legal and regulatory scrutiny. Analyst consensus targets average **\$215.76** with a high of \$310 and a low of \$165. The stock trades at a **forward P/E of ~6x** against a fair value estimate of \$230, suggesting potential undervaluation relative to near-term earnings growth, with a notably low PEG ratio of 0.16.

Fair Value

Upside	62.9%
Fair Value	\$230.0

Financial Health

6.8/10

Growth Rating



8.3/10

Profitability Rating



5.4/10

Cash Flow Rating



Financial health is determined by ranking the company on over 100 indicators compared to other companies in its sector that operate in similar economic markets.

Valuation

Reporting Date	2023	2024	2025	2026	2027
Period Ending	31/12	31/12	31/12	31/12	31/12
Capitalization	\$10.4B	\$11.8B	\$13.6B	\$8.5B	\$8.5B
P/E Ratio	15.4	11.5	9.91	6.08	5.85
Div. Yield	0.53	0.45	0.37	0.57	-
Capitalization / Revenue	0.74	0.77	0.80	0.46	0.44
EV / Revenue	1.11	1.09	1.09	0.72	0.69
EV / EBITDA	8.99	7.89	7.27	4.95	4.80
EV / FCF	34.0	17.8	16.7	-	-
FCF Yield	3.96	8.08	7.11	-	-
Price / Book	1.72	1.79	1.90	1.14	-

- Forecast

Pro Tips

Tips that distill complex financial data into concise, actionable investment insights.

🦖 Management has been aggressively buying back shares

🦖 High shareholder yield

🦖 Trading at a low P/E ratio relative to near-term earnings growth

🦖 Valuation implies a strong free cash flow yield

🦖 Prominent player in the Healthcare Providers & Services industry

🦖 Cash flows can sufficiently cover interest payments

🦖 Has maintained dividend payments for 24 consecutive years

🦖 Analysts predict the company will be profitable this year

🦖 Profitable over the last twelve months

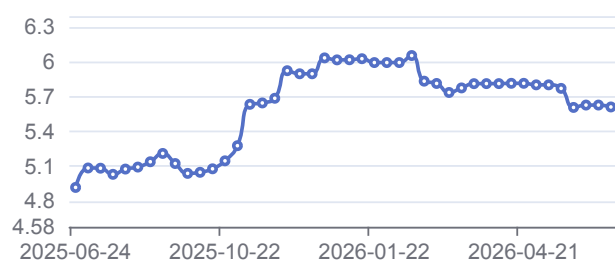
8 analysts have revised their earnings downwards for the upcoming period

Analyst Projections:

Analyst EPS Forecasts

Period Ending	Average	YoY Growth	Forward P/E	# of Analysts
2024	23.23	7.6%	6.1x	13
2025	24.13	8.4%	5.9x	13
2026	25.34	10.1%	5.6x	6

EPS Revisions Q2 2026



The chart above depicts the trend in analyst earnings per share (EPS) forecasts for the upcoming quarter. Analysts have reduced this quarter's expectations by 0.7% for EPS from \$4.99 per share to \$4.96 per share over the last 12 months.

EPS Revisions FY2026

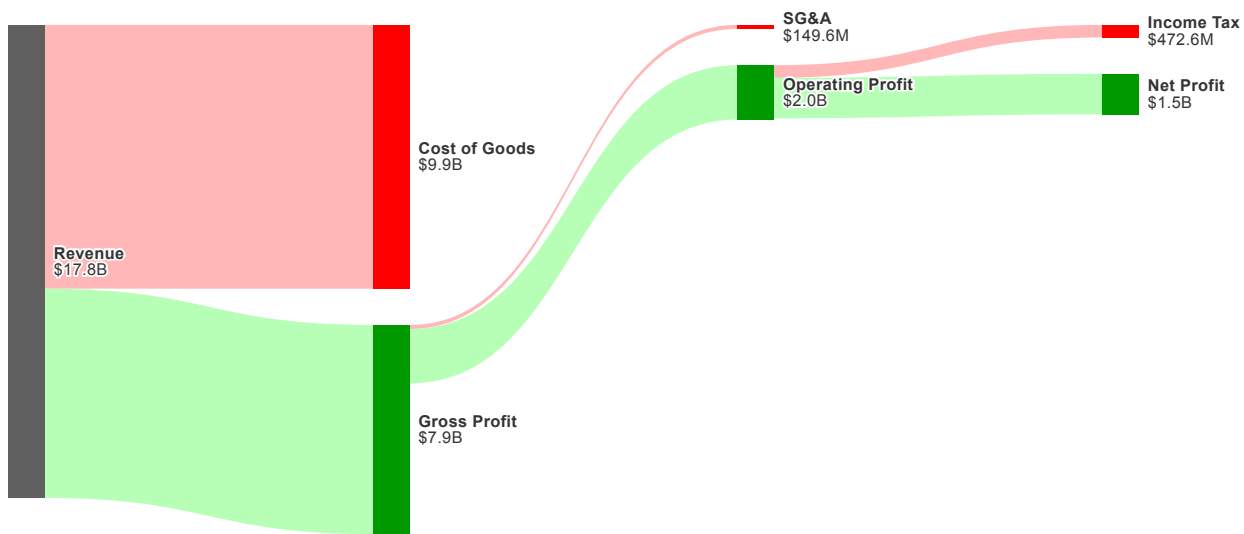


The chart above depicts the trend in analyst earnings per share (EPS) forecasts for the FY2025. Analysts have increased this quarter's expectations by 10.5% for EPS from \$19.70 per share to \$21.77 per share over the last 12 months. The company is expected to report earnings for Q2, 2026, on July 27, 2026.

Latest Ratings

Date	Analyst	Rating	Target
Jun 01, 26	Wells Fargo	Hold	\$165
May 29, 26	UBS	Buy	\$310
May 20, 26	JPMorgan	Hold	\$205
Apr 30, 26	UBS	Buy	\$310
Apr 29, 26	RBC Capital	Hold	\$190
Apr 29, 26	Morgan Stanley	Hold	\$212
Apr 29, 26	Baird	Hold	\$204
Apr 29, 26	Raymond James	Hold	N/A
Apr 28, 26	Mizuho	Buy	\$230
Apr 28, 26	TD Cowen	Buy	\$230
Apr 28, 26	Piper Sandler	Buy	\$205
Apr 28, 26	UBS	Buy	\$320
Apr 28, 26	JPMorgan	Buy	\$420
Apr 28, 26	Cantor Fitzgerald	Hold	\$229
Apr 28, 26	Barclays	Buy	\$238
Apr 15, 26	Cantor Fitzgerald	Hold	\$229
Apr 13, 26	Guggenheim	Buy	\$262
Apr 02, 26	Cantor Fitzgerald	Hold	\$229

Y LTM Financials:



* Income Statement is based on LTM data from 2025-03-31 to 2026-03-31

Income Statement

Date	2022	2023	2024	2025	LTM
Revenue	13,399	14,282	15,827	17,364	17,760
Operating Income	1,061	1,175	1,681	1,994	2,042
Net Income to Stockholders	675.6	717.8	1,142	1,488	1,520
Shares Outstanding	71.4	68.3	66.0	62.5	60.5
Diluted EPS	9.14	10.2	16.8	23.1	24.0
EBITDA	1,643	1,743	2,266	2,612	2,667

Balance Sheet

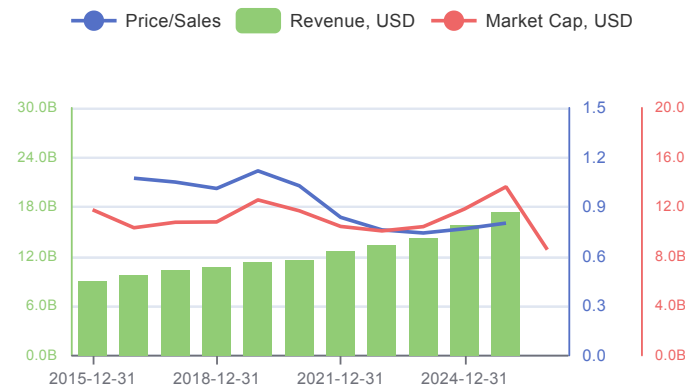
Date	2022	2023	2024	2025	LTM
Total Current Assets	2,537	2,811	2,816	3,407	3,499
Total Assets	13,494	13,967	14,469	15,527	15,681
Total Current Liabilities	1,914	2,013	2,210	3,239	3,244
Total Liabilities	7,524	7,765	7,706	8,118	8,076
Total Equity	5,970	6,201	6,762	7,409	7,604
Total Debt	5,271	5,366	4,955	5,166	5,125

Cash Flow Statement

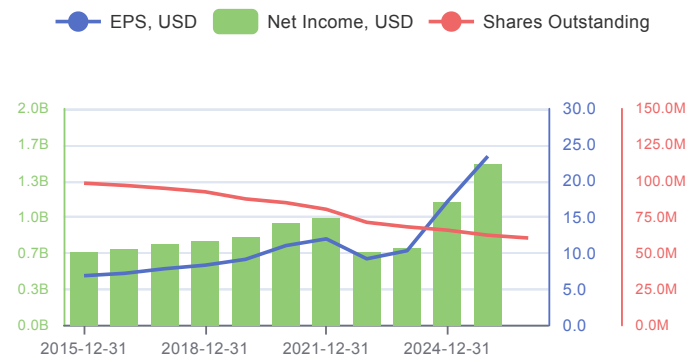
Date	2022	2023	2024	2025	LTM
Cash from Operations	996.0	1,267	2,067	1,864	1,906
Cash from Investing	-647.3	-763.3	-911.1	-1,070	-996.9
Cash from Financing	-318.4	-493.9	-1,144	-749.7	-880.2
Levered Free Cash Flow	0.0	0.0	0.0	0.0	0.0

*In USD millions, except number of shares, which are reflected in thousands, and per share amounts.

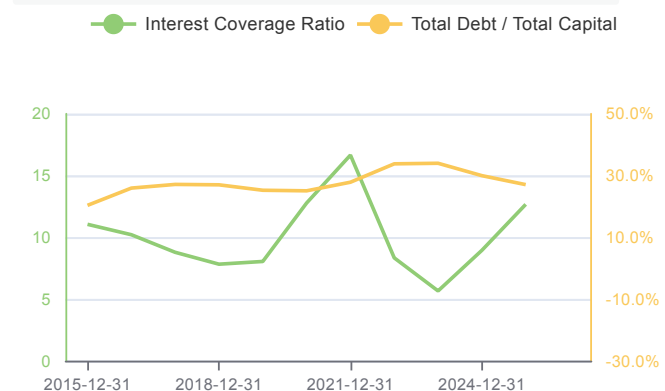
Revenue, Market Cap, Price/sales



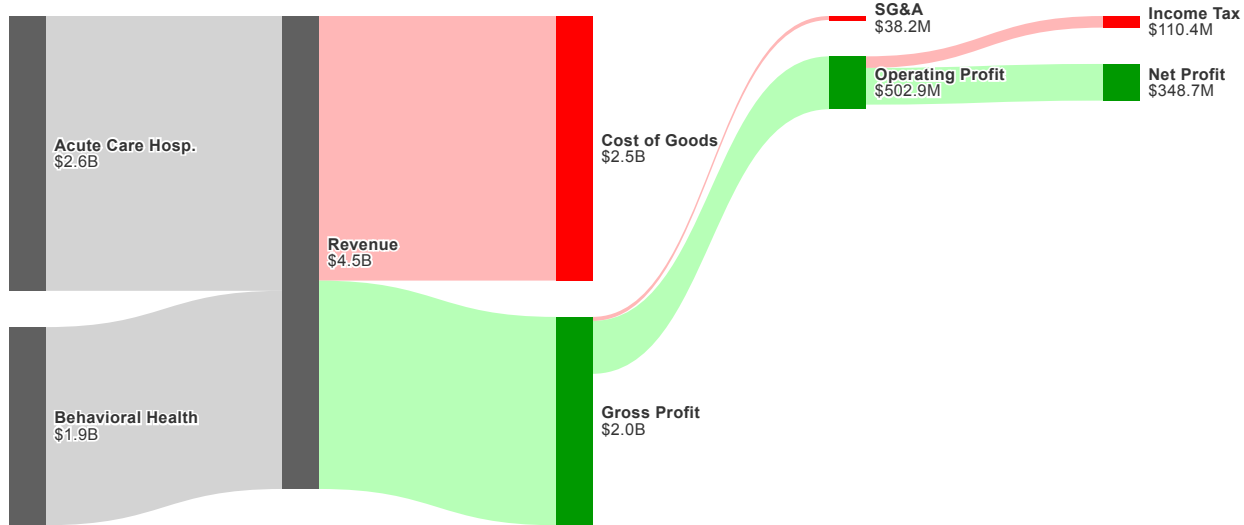
Net Income, EPS, Shares



Leverage and Debt



Q1 Financials



* Revenue segments are based on data from 2026-03-31

* Income Statement is based on LTM data from 2026-03-31

Income Statement

Date	Q1 2025	Q2 2025	Q3 2025	Q4 2025	Q1 2026
Revenue	4,099	4,283	4,495	4,486	4,495
Operating Income	454.8	500.3	521.7	517.2	502.9
Net Income to Stockholders	316.7	353.2	373.0	445.9	348.7
Shares Outstanding	65.0	64.4	63.6	62.5	61.1
Diluted EPS	4.80	5.43	5.86	7.06	5.65
EBITDA	603.2	652.3	676.7	680.6	658.3

Balance Sheet

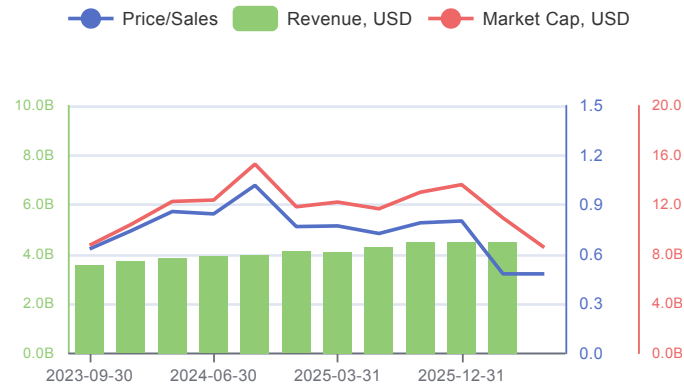
Date	Q1 2025	Q2 2025	Q3 2025	Q4 2025	Q1 2026
Total Current Assets	3,077	2,990	3,258	3,407	3,499
Total Assets	14,875	14,985	15,343	15,527	15,681
Total Current Liabilities	2,330	2,317	3,159	3,239	3,244
Total Liabilities	7,989	7,840	8,046	8,118	8,076
Total Equity	6,885	7,145	7,297	7,409	7,604
Total Debt	5,094	5,008	5,110	5,166	5,125

Cash Flow Statement

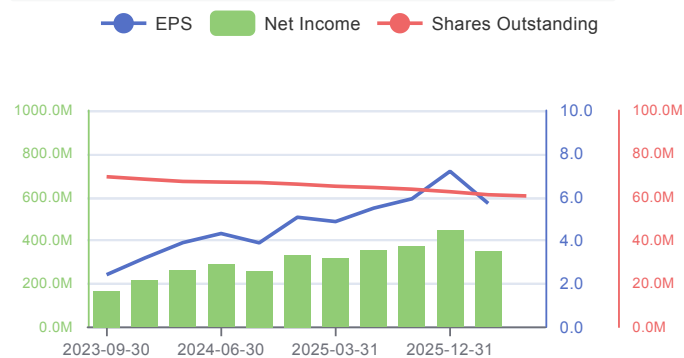
Date	Q1 2025	Q2 2025	Q3 2025	Q4 2025	Q1 2026
Cash from Operations	360.0	549.0	380.7	574.7	401.6
Cash from Investing	-271.3	-305.9	-269.4	-224.0	-197.6
Cash from Financing	-91.4	-227.7	-134.5	-296.1	-221.9
Levered Free Cash Flow	0.0	0.0	0.0	0.0	184.5

*In USD millions, except number of shares, which are reflected in thousands, and per share amounts.

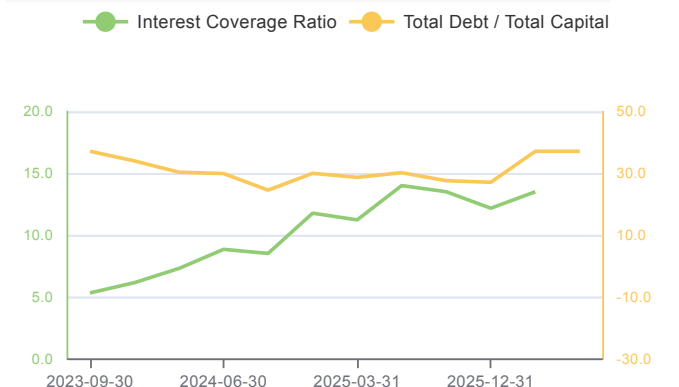
Revenue, Market Cap, Price/sales



Net Income, EPS, Shares



Leverage and Debt



Momentum & Technical Indicators

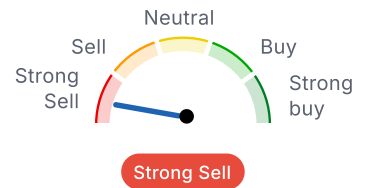
Price Momentum

Metric	UHS	Percentile	Score
Price % of 52 Week High	57.5%	45.6%	2.3
1 Week Price Total Return	-3.6%	23.8%	1.2
2 Week Price Total Return	-2.6%	38.6%	1.9
3 Week Price Total Return	-3.2%	50.7%	2.5
1 Month Price Total Return	-14.0%	18.1%	0.9
3 Month Price Total Return	-23.9%	17.5%	0.9
6 Month Price Total Return	-37.8%	18.5%	0.9
1 Year Price Total Return	-17.4%	38.9%	1.9
2 Year Price Total Return	-24.6%	45.0%	2.3
3 Year Price Total Return	-4.0%	64.1%	3.2
4 Year Price Total Return	38.8%	77.3%	3.9
5 Year Price Total Return	-1.3%	78.2%	3.9

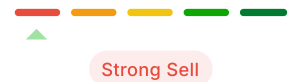
Peers

CHE	THC
77.9%	69.8%
4.0%	-1.2%
-1.2%	6.5%
2.3%	-1.6%
-1.1%	-7.9%
15.6%	-13.9%
0.5%	-12.1%
-20.1%	2.7%
-17.5%	27.6%
-18.4%	120.1%
-2.0%	225.9%
-5.6%	161.7%

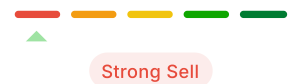
Technical Summary



Moving Averages



Technical Indicators



The Moving Average Score is based on various moving averages, both simple and exponential, with ranges from 5 to 200.

The Technical Score is calculated based on key technical indicators, including RSI, Stochastic, MACD, Williams %R, CCI, ATR, Highs/Lows, Ultimate Oscillator, ROC, and Bull/Bear Power, among others.

Peer Benchmarks:

Market and Yield Metrics

Metric	UHS	CHE	THC
Market Cap	\$8.5B	\$5.8B	\$14.9B
Price % of 52 Week High	57.3%	77.5%	69.8%
Div Yield	0.57%	0.55%	0%
Beta	1.08	0.54	1.28
1 Year Return	-17.4%	-20.1%	2.71%

Growth Metrics

Metric	UHS	CHE	THC
Revenue Growth	10.4%	2.07%	4.51%
Revenue CAGR (5y)	8.48%	4.00%	3.85%
Net Income Growth	27.1%	-15.9%	17.0%
Net Income CAGR (5y)	9.54%	-3.65%	28.7%
Revenue Forecast CAGR (5y)	6.12%	4.70%	4.09%
Net Income Forecast CAGR (5y)	0.27%	8.99%	0.38%

Financial Statement Metrics

Metric	UHS	CHE	THC
Revenue	\$17.8B	\$2.5B	\$21.5B
Gross Profit	\$7.9B	\$822.5M	\$8.9B
Operating Income	\$2.0B	\$333.7M	\$3.7B
Gross Profit Margin	44.2	32.4	41.3
Net Income to Common	\$1.5B	\$259.8M	\$1.7B
ROE	21.3	25.6	37.9
ROI	12.7	21.0	13.5
ROA	10.1	15.9	8.77
Total Assets	\$15.7B	\$1.5B	\$31.2B
Total Debt	\$5.1B	\$236.9M	\$13.2B

Valuation Metrics

Metric	UHS	CHE	THC
P/E Ratio (LTM)	5.62	22.2	8.73
PEG Ratio	0.16	-2.05	0.33
Price / Book	1.14	6.80	3.09
Price / LTM Sales	0.48	2.27	0.69
Analyst Upside	50.2%	1.54%	42.0%
Fair Value Upside	62.9%	2,000.0%	3,661.0%

Latest Insights

Warren AI

Bull Case

- Q1 2026 results beat both EPS (\$5.62 vs. \$5.44 est.) and revenue (\$4.49B vs. \$4.39B est.) forecasts, with 16.1% EPS growth and 11.7% operating cash flow growth year-over-year, demonstrating strong execution.
- The Talkspace acquisition accelerates UHS's end-to-end behavioral health continuum strategy, expected to be earnings-accretive within 12 months and reach a single-digit EBITDA multiple by year three, with bidirectional revenue synergy opportunities.
- UHS trades at a forward P/E of ~6x and a PEG ratio of 0.16, well below fair value estimates of ~\$230, suggesting significant upside potential for value-oriented investors.
- Management has \$1.3 billion in share repurchase authorization remaining and targets \$800–\$900 million in buybacks for '26, supported by strong free cash flow yield and 24 consecutive years of dividend payments.
- New capacity additions—including a 156-bed Florida hospital, 178 new beds in existing facilities, and two de novo behavioral health hospitals—are expected to drive meaningful EBITDA ramp in the back half of '26.
- Eight AI solutions deployed in revenue cycle operations are already yielding benefits in denials management and efficiency, with additional clinical AI initiatives planned for '26 that are expected to be incrementally margin-accretive.

Bear Case

- Raymond James downgraded UHS to Market Perform, noting the low end of EBITDA guidance implies ~16% normalized YoY growth ex-DPP following a quarter of 12% normalized EBITDA decline, raising attainability concerns for Q2–Q4 '26.
- Core EBITDA growth, excluding ~\$120–\$130 million in Medicaid supplemental payments and seasonal headwinds, was in the low single digits in Q1 '26, with Mizuho estimating base adjusted EBITDA ex-DPP declined ~3%.
- The company faces a \$75 million pre-tax HIX headwind in '26 from ACA subsidy expirations, with exchange-adjusted admissions declining ~5% in Q1 and the effective decline estimated at 10–12% after premium non-payment reserves.
- Medicaid supplemental payments (DPP) are projected to decrease by \$360–\$400 million annually from 2028–2032, representing a significant structural revenue headwind that will require substantial organic growth to offset.
- The stock has declined ~19% over the past 12 months and ~29% year-to-date, trading near its 52-week low, with technical indicators signaling a Strong Sell and 6 of 9 EPS revisions in the past 90 days trending downward.
- Mass tort litigation (130+ lawsuits alleging abuse at psychiatric facilities), DOJ fraud investigations in Nevada and California, and the executive departure of the Behavioral Health President add legal, regulatory, and operational risk.

Additional Insights

- UHS expanded its credit facility by \$900 million in April '26, including a \$400 million delayed draw term loan earmarked for Talkspace, bringing total debt to ~\$5.17 billion and leverage from just under 2x to just over 2x—management views this as manageable.
- Mizuho lowered its price target to \$230 from \$267 (maintaining Outperform) and TD Cowen lowered its target to \$230 from \$245 (maintaining Buy), reflecting reduced EBITDA estimates but continued confidence in long-term value.
- California nurse staffing ratio requirements effective June 1, '26 are expected to incur ~\$30 million in additional annual costs beyond '26; management states it remains on track with assumptions embedded in '26 guidance.
- Behavioral health labor cost growth moderated to ~6% in Q1 '26 from 7–8% in '25, with further moderation expected as hiring slows and turnover improves—a positive trend for margin recovery in the segment.
- The pending Florida Medicaid supplemental program approval (estimated ~\$50 million benefit, potentially higher) was not included in Q1 results and could provide an upside catalyst when recognized in a future quarter.

SWOT Analysis

WarrenAI

Strengths

- Market leader in U.S. private behavioral health inpatient services, controlling ~20% of the market with significant pricing power due to national inpatient psychiatric bed shortages.
- Diversified portfolio of 400+ facilities across acute care and behavioral health in the U.S., U.K., and Puerto Rico, providing revenue stability and geographic risk mitigation.
- Strong profitability metrics: 43.9% gross margin, 21.3% return on equity, and an adjusted EBITDA margin that expanded 160 bps to 14.9% in Q3 '25.
- Active AI deployment across eight revenue cycle use cases, with clinical AI initiatives underway, supporting long-term margin improvement and operational efficiency.
- Miller family holds 91.3% voting control, enabling long-term strategic decision-making without short-term market pressure.

Weaknesses

- Approximately 45% of revenue derived from Medicare and Medicaid, limiting negotiating leverage and exposing the company to government reimbursement policy changes.
- Free cash flow declined 22% year-over-year through Q3 '25, raising questions about sustainability of aggressive capital return programs.
- Core EBITDA growth in Q1 '26 was heavily reliant on Medicaid supplemental payments; excluding DPP, base EBITDA growth was negative, highlighting dependence on policy-driven benefits.
- Persistent labor shortages in nursing and psychiatric staff drive elevated wage inflation; behavioral health salaries and wages rose ~8% in '25, pressuring segment margins.
- Acute care same-facility adjusted admissions were flat in Q1 '26, reflecting volume vulnerability to seasonal and payer-mix headwinds.

Opportunities

- The U.S. behavioral health market is projected to grow to \$159 billion by 2035 (5.1% CAGR), providing a substantial long-term runway for UHS's core segment.
- Talkspace acquisition creates the industry's first end-to-end behavioral health continuum, opening new outpatient revenue streams, virtual IOP programs, and bidirectional referral synergies.
- Pending Florida Medicaid supplemental program approval (~\$50 million or higher benefit) represents a near-term upside catalyst not yet reflected in guidance.
- Aging baby boomer demographics are expected to drive increased demand for high-acuity acute care services including cardiology, orthopedics, and neurology—service lines already showing positive trends.
- Continued AI deployment in clinical operations is expected to incrementally improve margins and patient experience over time, with Hippocratic AI partnerships already in the '26 roadmap.

Threats

- Medicaid supplemental (DPP) payments projected to decline \$360–\$400 million annually from 2028–2032, representing a major structural revenue headwind requiring significant organic growth to offset.
- DOJ West Coast healthcare fraud strike force is actively investigating billing practices in Nevada and California—key UHS markets—with potential risk of Medicare accreditation loss described as an existential threat.
- Mass tort litigation including 130+ lawsuits alleging abuse at psychiatric facilities and a \$360 million jury award in '24 create substantial financial and reputational risk.
- ACA subsidy expiration is expected to result in a \$75 million pre-tax earnings headwind in '26, with exchange volume declines expected to steepen throughout the year.
- Intense competition from HCA, Tenet, and specialized behavioral health providers, combined with the broader industry shift toward outpatient and digital-first care, threatens inpatient volume growth and pricing power.

Earnings Call - Q1 2026

04/28/26 | WarrenAI

- Q1 '26 revenue grew 9.6% YoY to \$4.49B and adjusted EPS grew 16.1% to \$5.62, both exceeding consensus estimates, with operating cash flow of \$402 million up 11.7%.
- Talkspace acquisition announced March 9 is expected to be earnings-accretive within 12 months and reach a single-digit EBITDA multiple by year three, creating an end-to-end behavioral health continuum.
- Management reiterated full-year '26 guidance, including core EBITDA growth of ~5% ex-DPP, with earnings expected to ramp in Q2–Q4 driven by new facility openings, behavioral volume recovery, and wage moderation.

Bullish Highlights

- Acute care same-facility revenue rose 8.2% YoY; behavioral health same-facility revenue increased 7.3% with adjusted patient days up 1.6%, demonstrating broad-based segment growth.
- New capacity additions—156-bed Florida hospital opening in May, 178 beds across existing facilities in Q2, and two de novo behavioral health hospitals—are expected to drive back-half '26 EBITDA acceleration.
- Contract labor as a percentage of acute care revenues fell 40 bps YoY to 2.3%, and behavioral health wage growth moderated to ~6% from 7–8% in '25, signaling improving cost trends.
- March volumes showed more normative growth after weather and flu impacts were concentrated in January and February, suggesting Q2 volume recovery.
- Florida Medicaid supplemental program approval (~\$50 million benefit, potentially higher) is expected imminently and will be recorded as an upside revision to guidance when approved.

Q&A Highlights

- CFO Filton confirmed that all DPP and seasonal headwinds in Q1 were within internal expectations and embedded in full-year guidance; the company still expects to achieve ~5% core EBITDA growth for '26.
- On Talkspace, CEO Miller highlighted the opportunity to introduce 6,000 Talkspace clinicians into UHS's environment to develop virtual IOPs and improve step-down care continuity post-discharge.
- Management indicated buyback target of \$800–\$900 million for '26 remains intact despite the Talkspace acquisition, as leverage only moves from just under 2x to just over 2x.
- On behavioral health demand, management noted strong underlying demand but an accelerating shift toward outpatient delivery, which UHS is addressing through Thousand Branches, freestanding clinics, and Talkspace.
- On HIX, management stated the 25–30% full-year exchange volume decline assumption remains intact, with Q1 at ~10–12% effective decline and steepening expected through the year.

- Eight AI solutions deployed in revenue cycle operations are yielding measurable benefits; '26 roadmap includes clinical AI use cases developed with Hippocratic AI.
- Company repurchased 675,000 shares for \$127 million in Q1 '26, with \$1.3 billion remaining in buyback authorization and a target of \$800–\$900 million for the full year.

Bearish Highlights

- Excluding ~\$46 million of out-of-period DPP and seasonal headwinds, core acute EBITDA growth was in the low single digits; one analyst calculated core EBITDA was down ~\$30 million or ~5–6% YoY.
- HIX exchange-adjusted admissions declined ~5% in Q1, with an effective decline of ~10–12% after reserving for patients expected to lose coverage due to premium non-payment; \$75 million full-year headwind reiterated.
- Behavioral health salary and wage expense per adjusted patient day rose ~6% YoY, and California nurse staffing ratio requirements effective June 1 add ~\$30 million in annual costs beyond '26.
- Cedar Hill (D.C.) facility improvement is now expected to be more back-end loaded than originally contemplated, reducing near-term EBITDA contribution from new capacity.
- Payer mix showed slight increases in uninsured and slight decreases in Medicaid utilization, consistent with broader coverage erosion trends expected to continue through '26.

Misses

- Core EBITDA growth ex-DPP and seasonal items was below the ~5% full-year target in Q1, with management acknowledging the company was 'not at that core 5% growth' in the quarter.
- Behavioral health adjusted patient days grew only 1.6% in Q1, at the low end of the 2–3% volume guidance range even after adjusting for weather, suggesting demand capture challenges persist.
- The company did not provide a specific numerical bridge from Q1 core EBITDA to full-year guidance, leaving analysts to independently calculate a significant implied ramp that some found unconvincing.
- No update on California Medicaid supplemental program approval was provided; management described it as 'much less certain' than Florida, leaving a potential upside catalyst unquantified.
- Free cash flow trajectory and the 22% YoY decline through Q3 '25 were not directly addressed in the call, leaving questions about sustainability of the \$800–\$900 million buyback target unanswered.

Top News, last 60 days:

Universal Health Services Q1 2026 earnings beat expectations with 16.1% EPS growth, announces Talkspace acquisition

April 28, 2026

- UHS reported Q1 2026 EPS of \$5.62, exceeding forecast of \$5.44, and revenue of \$4.49 billion versus \$4.39 billion forecast, with stock rising 2.5% in premarket trading on Tuesday, 20260428.
- Revenue increased 9.6% yearoveryear and EPS grew 16.1% compared to Q1 2025, with operating cash flow of \$402 million up 11.7% from Q1 2025.
- Acute care samefacility revenue increased 8.2% yearoveryear, with revenue per adjusted admission up 6.3% including approximately \$30 million in prior period Nevada supplemental program benefits.
- Behavioral health samefacility revenue rose 7.3% with revenue per adjusted patient day up 5.8% and adjusted patient days up 1.6% yearoveryear.
- Company estimates Q1 2026 volumes were impacted by approximately 200 basis points in acute care and 4050 basis points in behavioral health due to weather and weaker flu/respiratory activity.

Importance - 6/10

Neutral

Universal Health Services Expands Credit Facility by \$900 Million, Adds New Loans for Talkspace Acquisition

April 24, 2026

- Universal Health Services entered into an Eleventh Amendment to its credit agreement on Wednesday, April 22, 2026, expanding its senior secured credit facility by up to \$900 million.
- The expansion includes a \$200 million incremental revolving credit line, a \$300 million incremental tranche A term loan, and a \$400 million delayed draw tranche A term loan, all maturing September 26, 2029.
- Proceeds from the \$400 million delayed draw term loan are intended to fund the previously announced acquisition of Talkspace, Inc.
- The company has a market capitalization of \$11.06 billion, total debt of approximately \$5.17 billion, and a debtoequity ratio of 0.71.
- Interest margins will be based on consolidated net leverage ratio, initially set at 0.25% for ABR loans and 1.25% for term benchmark and RFR loans.

Importance - 6/10

Neutral

Universal Health Services Hits 52-Week Low, Faces Multiple Headwinds

May 28, 2026

- UHS stock traded at \$151.90 on Thursday, May 28, 2026, near its 52week low of \$152.33, with stock value down 19.24% over the past 12 months and yeartodate losses of 29%.
- The company declared a \$0.20 per share cash dividend payable June 18, 2026, to shareholders of record as of June 8, 2026.
- Matthew J. Peterson, Executive VP and President of Behavioral Health, will resign effective June 19, 2026, resulting in termination of all unvested stock options and restricted stock units.
- Raymond James downgraded UHS to Market Perform from Outperform due to EBITDA guidance concerns, while Bank of America's report showed Q2 foot traffic data fell below expectations.

Importance - 5/10

Negative

Raymond James Downgrades Universal Health Services to Market Perform on Q2-Q4 Guidance Concerns

April 29, 2026

- Raymond James downgraded Universal Health Services (NYSE:UHS) to Market Perform from Outperform on Tuesday, April 29, 2026, citing concerns about the attainability of the company's secondquarter through fourthquarter EBITDA guidance.
- The stock fell to \$162.54, down nearly 10% over the past week and trading just 7% above its 52week low of \$152.33.
- Raymond James noted the low end of EBITDA guidance implies approximately 16% yearoveryear normalized growth excluding DPP following a quarter of 12% normalized EBITDA decline.
- Universal Health Services reported firstquarter 2026 earnings per share of \$5.62, surpassing analyst estimates of \$5.41, and revenue of \$4.49 billion versus anticipated \$4.39 billion.
- Mizuho adjusted its price target to \$230 from \$267, and TD Cowen lowered its price target from \$245 to \$230 while maintaining a Buy rating.

Importance - 5/10

Negative

[Raymond James Downgrades Universal Health Services to Market Perform on Concerns About Attainability of Rest-of-Year EBITDA Guidance](#)

April 29, 2026

- Raymond James downgraded Universal Health Services (UHS) from Outperform to Market Perform due to concerns about the attainability of 2Q4Q EBITDA guidance.
- The low end of EBITDA guidance implies approximately 16% yearoveryear normalized growth excluding DPP following a quarter of 12% normalized EBITDA decline.
- Analysts believe the market will be unconvinced about the implied inflection absent more support.

Importance - 5/10

Negative

[Mizuho Lowers Universal Health Services Price Target to \\$230 from \\$267, Maintains Outperform Rating](#)

April 28, 2026

- Mizuho lowered its price target on Universal Health Services (UHS) to \$230 from \$267 while maintaining an Outperform rating.
- Mizuho reduced its 2026E/2028E adjusted EBITDA estimates by 2% to account for negative impact of seasonal headwinds in Q1:26.
- UHS stock declined 9% postQ1:26 results due to concerns about base EBITDA growth, with Mizuho estimating base adjusted EBITDA excluding incremental Medicaid DPP, weather and other factors declined by 3%.
- Mizuho's new \$230 price target reflects an EV/EBITDA of 7.1x 2026E adjusted EBITDA, with the reduction driven by lower earnings from seasonal headwinds and a lower valuation multiple of 7.0x versus 8.0x previously.

Importance - 5/10

Neutral

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